SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Cetuximab + cisplatin + fluorouracil

Treatment of locally advanced or metastatic squamous cell cancer of the head and neck. · NCCP 00417 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Cetuximab with cisplatin and fluorouracil is used to treat squamous head and neck cancer. Discuss the individual aim, likely benefit, symptom control and alternatives with your oncology team.

Schedule
Cisplatin is given on Day 1, fluorouracil on Days 1–4, and cetuximab on Days 1, 8 and 15 of each 21-day cycle, for up to 6 combination cycles. The team may then consider weekly cetuximab alone under a separate maintenance plan (NCCP 00207). This is not an automatic continuation.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Cetuximab
EGFR-TARGETED ANTIBODY

- ☐ Infusion reactions, including anaphylaxis
- ☐ Acne-like rash, dry or cracked skin and nail infections
- ☐ Low magnesium and other mineral changes
- ☐ Eye inflammation
- ☐ Diarrhoea and dehydration
- ☐ Rare serious lung inflammation

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Nausea and vomiting
- ☐ Kidney injury and mineral changes
- ☐ Hearing loss or tinnitus, which can persist
- ☐ Nerve damage, which can persist
- ☐ Infusion reaction and effects on fertility

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss the individual aim, expected benefit, alternatives and uncertainties; common, serious and lasting effects; infusion reactions and infection; fertility and pregnancy; treatment burden; support medicines and urgent contacts. For a locally advanced treatment plan, confirm whether cetuximab use falls outside the current licensed combination indication and discuss this explicitly.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00417 v7.

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